

Branch Leader Review — Evaluating Nurse Territories

A quarterly read on demand, capacity, and coverage. The ED overlays three months of trends per nurse and territory, then decides where to redraw zips, staff up, or rebalance. Example: Charleston Tri-County, 4 SN territories.

THE 3-MONTH PICTURE — THE SIGNALS SHE PULLS FIRST

Referral distribution

Shifting West / South

New-referral volume migrating to West Ashley + Johns Island growth corridors; peninsula flat-to-down.

Per-diem utilization

▲ 40% · 140 hrs/mo

Premium coverage climbing 3 months straight — a structural signal, not a one-off spike.

Caseload spread

1 over · 1 under

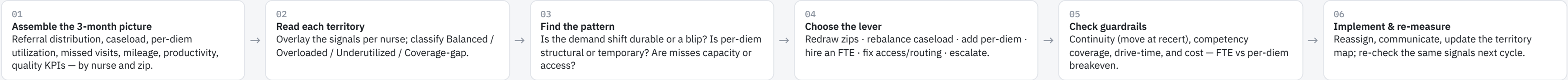
Imbalanced: Torres 112% (over ceiling), Blake 78% (slack). Room to rebalance internally.

Missed-visit rate

5.4% branch · 9.2% peak

Rising, concentrated in Downtown/peninsula — access & geography, not raw capacity.

THE EVALUATION FLOW



TERRITORY DIAGNOSIS

West Ashley — RN Torres

29407 · 29414

OVERLOADED

CASELOAD112%

REFERRALS 3-MO▲ 18%

PER-DIEM62 hrs ▲

MISSED6.8% ▲

OVERTIME▲

RECOMMENDED LEVER

Shed a zip + queue an FTE. Move 29414 to Blake to pull her under ceiling now. The rising per-diem here is paying premium for **durable** demand growth — earmark this territory for the new hire.

Mt Pleasant / East — RN Blake

29464 · 29466

UNDERUTILIZED

CASELOAD78%

REFERRALS 3-MOflat

PER-DIEM8 hrs

MISSED2.1%

MILEAGElow

RECOMMENDED LEVER

Absorb 29414 from Torres. Clear slack and clean reliability — the best home for the rebalance, and positioned to take rising east-side referrals without new cost.

Downtown / Peninsula — RN Diaz

29401 · 29403

COVERAGE / RELIABILITY GAP

CASELOAD96%

REFERRALS 3-MO▼ slight

PER-DIEM40 hrs ▲

MISSED9.2% ▲

MILEAGEhigh

RECOMMENDED LEVER

Fix access, not headcount. Misses are no-access + parking, not overload — add day-before confirmation discipline and tighten the route; a downtown-only per-diem for surge. No redraw yet — watch one more cycle.

Johns Island / Rural — RN Ellis

29455 · 29487

GROWING · DRIVE-TIME RISK

CASELOAD88%

REFERRALS 3-MO▲ 14%

PER-DIEM30 hrs ▲

MISSED4.5%

MILEAGEvery high

RECOMMENDED LEVER

Base the new FTE south. Rural demand rising with long drives; a satellite per-diem or the new hire based on Johns Island caps mileage and per-diem. Guard continuity as the territory grows.

BRANCH DECISION & GUARDRAILS

The signals converge: demand is migrating west and south, per-diem is up 40% and structural, one nurse is over and one is under, and peninsula misses are an access problem — not a staffing one. So:

- Redraw — move zip 29414 from Torres to Blake to level the over/under today.
- Hire 1 FTE RN, based south — three months of climbing per-diem in the West Ashley + Johns Island growth corridors is cheaper to internalize than to keep renting; model the FTE-vs-per-diem breakeven before posting.
- Peninsula = process fix — confirmation calls + route tightening for Diaz, plus a downtown per-diem for surge. No headcount.
- Guardrails — phase patient moves at recert to protect continuity; keep the wound-competent RN on West Ashley; hold drive-time under target; don't push Blake past ceiling.

Re-measure next cycle: per-diem hours, missed-visit rate by zip, caseload balance, and mileage — did the redraw + hire bend the curves?